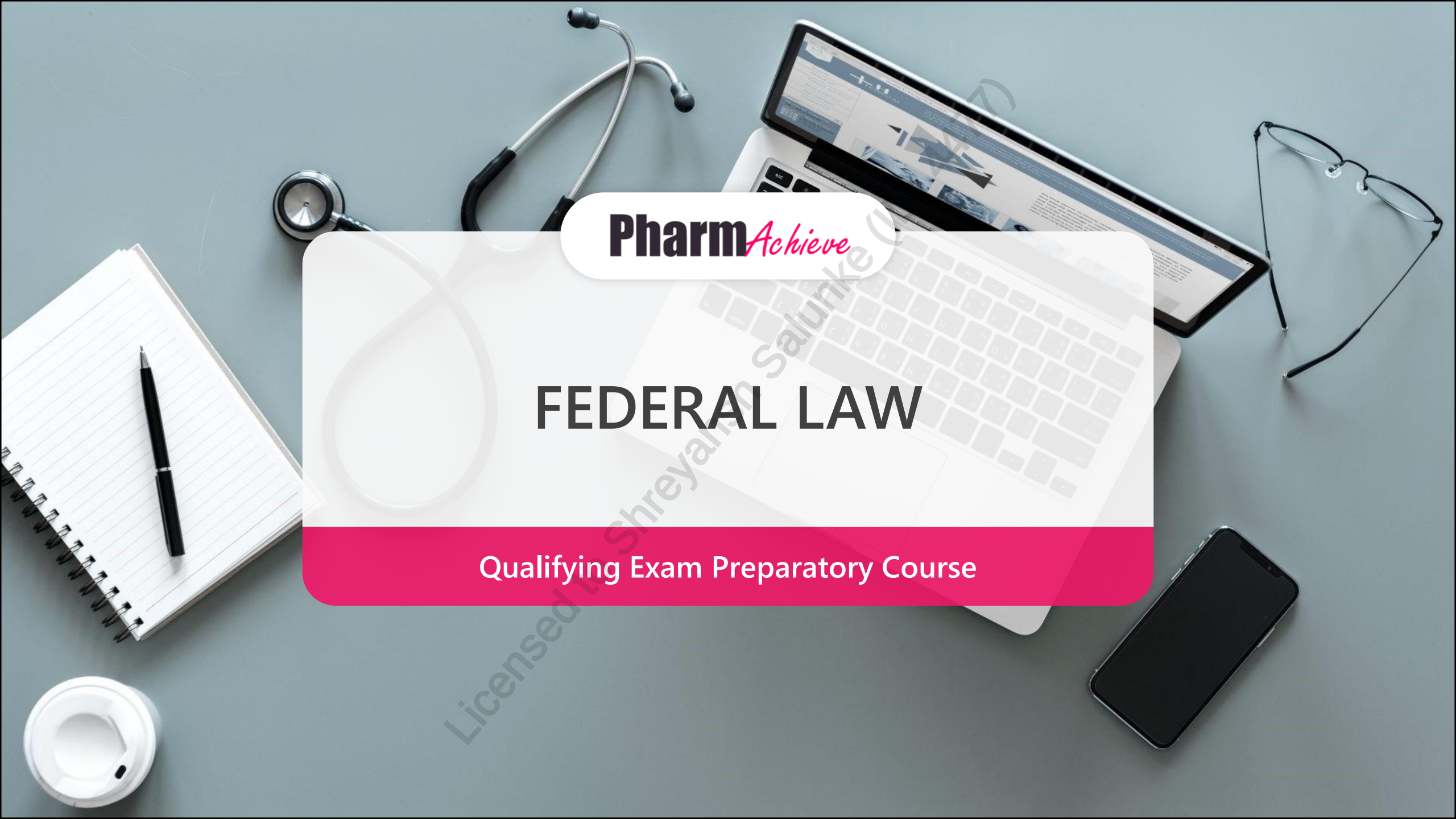




Pharm*Achieve*

FEDERAL LAW

Qualifying Exam Preparatory Course

COMPETENCIES COVERED IN THIS PRESENTATION...

Providing Care:
Clinical Care

Providing Care:
Distribution

Knowledge
& Expertise

Communication
& Collaboration

Leadership &
Stewardship

Professionalism

LEGEND

CDSA Controlled Drugs and Substances Act

F&DA Food and Drugs Act

MMT Methadone Maintenance Treatment

NAPRA National Association of Pharmacy Regulatory
Authorities

NDSAC National Drug Scheduling Advisory Committee

PDL Prescription Drug List

PIPEDA Personal Information Protection And Electronic
Document Act

P/T Province or Territory

PEBC Pharmacy Examining Board of Canada

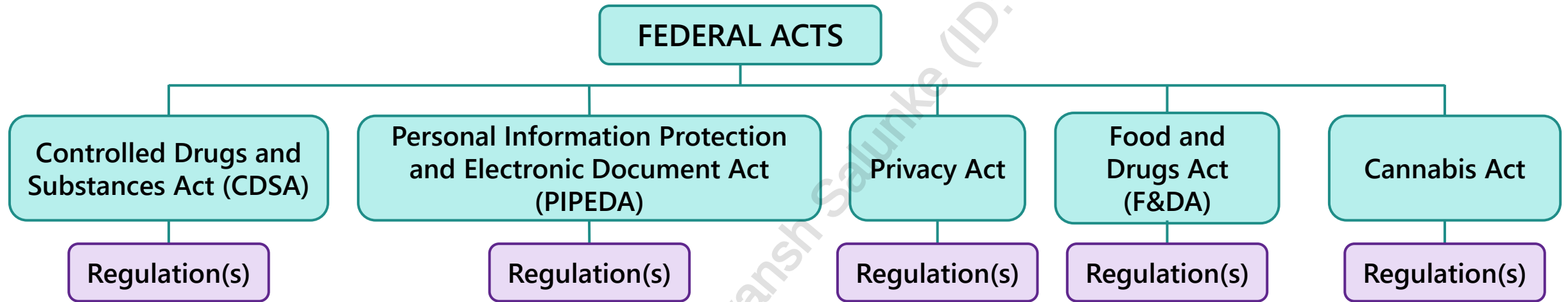
R_x Prescription

SDM Substitute Decision Maker

THC Tetrahydrocannabinol

5 KEY FEDERAL ACTS AND REGULATIONS

Acts are laws created by parliament that define what acts can or cannot be done



REGULATIONS:

- Often many regulations correspond to a single act
- Like laws, they limit what you can or cannot do
- Each act specifies who can issue regulations for that Act (usually a government agency's sponsoring minister but it could be other bodies of government. E.g. If Health Canada would issue the regulations, it will say "Minister of Health")
- Regulations can be changed by a Minister without a new law being passed so they are much easier to change and tend to change more frequently than laws

FOOD AND DRUGS ACT (F&DA)

- Canada's federal legislation regarding the production, import, export, transport, and sale of food, drugs, medical devices, and cosmetics
- The F&DA gives the Federal Minister of Health the ability to:
 - Regulate
 - Inspect
 - Seize
 - Impose Penalties

On drugs, foods, cosmetics, and medical devices

FOOD AND DRUGS ACT (F&DA)

SCHEDULES

The F&DA has 4 schedules:

Schedule A

Diseases for which treatments may not be promoted to public e.g. acute alcoholism, acute anxiety state, asthma, cancer

Schedule B

Publications describing official drug standards e.g. The Canadian Formulary, The National Formulary

*Any substance/drug sold in Canada must meet the criteria outlined in the standards

Schedule C

Radio-pharmaceuticals

Schedule D

Allergenic substances, blood derivatives, drugs obtained by recombinant DNA, vaccines, insulin, and other biologicals

FOOD AND DRUGS ACT (F&DA)

REGULATIONS

Part A: Administration

- Contains information on prohibitions, powers (authority), definitions, and obligations

Part B: Food

- Contains information on general labelling, nutrition labelling, standards of composition, food additives, food packaging materials, etc.

Part C: Drugs (Have a **Pr** symbol on label)

- **Prescription Drug List (PDL):** List of medicinal ingredients that if found in a drug, requires a prescription for sale in Canada – formerly known as schedule F drugs. PDL does NOT include drugs that are regulated under CDSA (e.g. narcotics)

FOOD AND DRUGS ACT (F&DA)

REGULATIONS

Part D: Vitamins, minerals, and amino acids

- Identifies foods that must or can be fortified with vitamins and minerals. Also specifies the vitamins and the quantity that are permitted in foods

Part E: Cyclamate and saccharin sweeteners

- Regulates labelling and advertising of cyclamate and saccharin artificial sweeteners

Part G: Controlled drugs (have a symbol on label)

- Contains information regarding the possession, sale, production, and transportation of controlled drugs such as stimulants, barbiturates, anabolic steroids. (narcotics, benzodiazepines, and other targeted substances are regulated under CDSA)

Part J: Restricted drugs

- Contains information regarding the possession, production, sale, transportation, and importation of restricted drugs for clinical trials or research purposes

CONTROLLED DRUGS AND SUBSTANCES ACT (CDSA)

- Federal legislation regarding the import/export, production, distribution, and use of substances that can alter mental processes and produce harm when distributed without supervision – *strictest of all acts*

Precursor Control Regulations

- Regulates the sale, purchase, possession and use of precursor chemicals
 - Class A precursor: Acetic anhydride, ephedra, ephedrine, ergotamine, and others
 - Class B precursor: Acetone, ethyl ether, hydrochloric acid, sulfuric acid, toluene

PRESCRIBING AUTHORITY

CONTROLLED BY THE CDSA

Who is permitted to prescribe controlled substances as per the New Classes of Practitioners Regulations?

- ✓ Doctors of medicine
- ✓ Dentists
- ✓ Doctors of veterinary medicine
- ✓ Midwives
- ✓ Podiatrists
- ✓ Nurse Practitioners

Exclusions of drugs practitioners cannot prescribe:

Midwife or Podiatrist:

- Anabolic steroids
- Methadone
- Buprenorphine

Nurse Practitioners:

- Anabolic steroids (except testosterone)
- Opium
- Coca leaves

Only as permitted by provincial regulations! Meaning they must be registered and able to practice as a doctor, dentist, midwife, etc. under the laws of each province

CONTROLLED DRUGS AND SUBSTANCES ACT (CDSA) SCHEDULES

- **Schedules I to VIII:** NOT currently used in practice, and are only used to assign penalties for product possession, trafficking, and manufacturing

Schedule I	Opium poppy, amphetamines, narcotics, cocaine, and others
Schedule II	Synthetic cannabinoid receptor type 1 agonists, its salts, derivatives and others
Schedule III	Methylphenidate and others
Schedule IV	Barbiturates, anabolic steroids, benzodiazepines, and others
Schedule V	Prophylhexedrine (Repealed)
Schedule VI	Ephedrine, ergometrine, ergotamine, and others
Schedule VII	Cannabis resin 3 kg, cannabis 3 kg (Repealed)
Schedule VIII	Cannabis over 30 g, cannabis resin 1 g (Repealed)

NARCOTIC CONTROL REGULATIONS (NCR)

- Narcotic Control Regulations (NCR) define a “narcotic” through a schedule - referred to as the *schedule to the NCRs of CDSA*
- All drugs in this schedule must have a (N) symbol on labelling as per F&DA

Types of narcotics:

1. Narcotic Drugs (Schedule N) commonly referred to as ‘written prescription narcotics’ or ‘reportable narcotics’
 2. Narcotic Preparations also known as ‘verbal prescription narcotics’
 3. Exempted Codeine Preparations also known as ‘low-dose codeine preparations’
- The classes have different rules for:
 - Purchase and sale record requirements
 - Prescription requirements
 - Rules around dispensing or sale to the patient

PRESCRIPTION REQUIREMENTS

Refills

- Total quantity written as single number is NOT required on a prescription because it can be calculated
- For example:
Crestor 20 mg, dispense 60 tablets, 5 refills
Thus, the total quantity can be calculated as follows: $60 \times 6 = 360$ tablets

Intervals

- Not required for narcotics but recommended for monitoring purposes
- If specified, pharmacists can only dispense the next refill/part-fill when appropriate amount of time has passed
- If patient presents early for a refill, doctor must be contacted
 - If doctor unavailable, pharmacists can use their professional judgment to fill or refuse and the reason should be documented

PRESCRIPTION REQUIREMENTS

Part-Fills

- The “total authorized quantity” must be written on the prescription
- The dispensed quantity with/without intervals must be specified
- Amount dispensed should be less than the total authorized quantity

E.g.:

Hydromorphone 2 mg

Total quantity: 200 tablets

Dispense 60 tablets every 2 weeks

NARCOTIC CONTROL REGULATIONS (NCR)

Narcotic drug defined as any product meeting at least 1 of the following criteria:

- Products containing only a single narcotic ingredient (eg. codeine tablets)
 - Product containing >1 narcotic
 - Injectable narcotics
 - All narcotic products containing <2 non-narcotic ingredients
 - All products containing hydrocodone, methadone, oxycodone, pentazocine and diacetylmorphine (heroin)
- ✓ Written, faxed R_x
 - ✓ Part-fills
 - ✗ Verbal R_x, refills, transfers
 - Purchase report required
 - Sales report required
 - E.g. morphine, methadone, oxycodone, Lenoltec #4

- Purchase and sales records must be kept for a minimum of 2 years
- Theft or loss must be reported within 10 days to Health Canada

NARCOTIC CONTROL REGULATIONS (NCR)

Narcotic preparations defined as any product containing only 1 narcotic ingredient AND ≥ 2 non-narcotic ingredients except products containing hydrocodone, methadone, oxycodone, pentazocine and diacetylmorphine (heroin)

- ✓ Written, fax, verbal R_x
- ✓ Part-fills
- ✗ Refills, transfers
- Purchase record required
- E.g. Lenoltec #3

Exempted codeine products defined as any product containing up to **8 mg** or its equivalent of codeine phosphate per tablet/unit (in other solid form) or not more than **20 mg** or its equivalent of codeine phosphate **per 30 mL** of liquid, AND ≥ 2 non-narcotic ingredients*

- ✓ Written, fax, verbal R_x
- ✓ Part-fills
- ✗ Refills, transfers
- Purchase record required
- E.g. Lenoltec #1

*Exempted codeine products may be sold without a prescription, as per conditions in Schedule II of NAPRA National Drug Schedules. When dispensed pursuant to a prescription, follow the same regulations as for Narcotic Preparations (Verbal Prescription Narcotics)

CONTROLLED DRUGS

- SCHEDULE G UNDER F&DA
- SUBDIVIDED INTO PARTS I, II, III

Part I controlled drugs are defined as any controlled drugs in Part I of schedule G
Products containing only 1 controlled drug *OR* combinations with > 1 controlled drug

Part I controlled preparations (**not available in Canada**) are defined as products containing 1 controlled drug in Part I and ≥ 1 non-controlled ingredients

- ✓ Written, fax, verbal R_x
- ✓ Written refills with intervals
- ✓ Part-fills
- ✗ Verbal refills, transfers

- Purchase and sales record required
- E.g. amphetamines

Part II controlled drugs are defined as any drug listed in Part II of schedule G, for example most barbiturates

Part II controlled preparations are defined as products containing 1 controlled drug in Part II and ≥ 1 non-controlled ingredients e.g. Tecnal®

- ✓ Written, fax, verbal R_x
- ✓ Refills with intervals
- ✓ Part-fills
- ✗ Transfers

- Purchase record required
- E.g. phenobarbital, butorphanol

CONTROLLED DRUGS

- SCHEDULE G UNDER F&DA
- SUBDIVIDED INTO PARTS I, II, III

Part III controlled drugs are defined as any drug listed in Part III of schedule G, for example anabolic steroids and derivatives

Part III controlled preparations (**not available in Canada**) are defined as products containing 1 controlled drug in Part III and ≥ 1 non-controlled ingredients

✓ Written, fax, verbal R_x

✓ Refills with intervals

✓ Part-fills

✗ Transfers

- Purchase record required

- E.g. testosterone

BENZODIAZEPINES AND OTHER TARGETED SUBSTANCES

Benzodiazepines and other targeted substances are identified with  e.g. diazepam, lorazepam, oxazepam, zolpidem

- ✓ Written, fax, verbal R_x
- ✓ Refills (written and verbal)
- ✓ ONE transfer only
- ✗ R_x NOT valid >1 year after issuance
- Purchase record required



Requirements for written verbal prescriptions (for any controlled substance)

- Name/initials of pharmacist
- Name/initials and address of practitioner
- Name and address of patient
- Name, form and quantity of substance
- Prescription number
- Date of which substance was dispensed
- Directions for use

LET'S RECAP...

- NARCOTICS
- CONTROLLED DRUGS
- BENZODIAZEPINES & TARGETED SUBSTANCES

DRUG TYPE	WRITTEN/ FAXED	VERBAL	PART-FILL	REFILLS	TRANSFERS	PURCHASE RECORD	SALES RECORD
STRAIGHT NARCOTICS	✓	✗	✓	✗	✗	✓	✓
NARCOTIC PREPARATIONS	✓	✓	✓	✗	✗	✓	✗
EXEMPTED CODEINE PRODUCTS	✓	✓	✓	✗	✗	✓	✗
PART I CONTROLLED DRUGS	✓	✓	✓	written with interval	✗	✓	✓
PART II CONTROLLED DRUGS	✓	✓	✓	with interval	✗	✓	✗
PART II CONTROLLED PREPARATIONS	✓	✓	✓	with interval	✗	✓	✗
PART III CONTROLLED DRUGS	✓	✓	✓	with interval	✗	✓	✗
BENZO & TARGETED SUBSTANCES	✓	✓	✓	✓	ONE	✓	✗

TEMPORARY CDSA SUBSECTION 56(1) CLASS EXEMPTION

Temporary Exemptions for Practitioners and Pharmacists due to COVID

(October 2020 - earliest of September 30, 2026, the date it is replaced by another exemption, or the date on which it is revoked):

- **Accept a verbal order** from a prescriber for a controlled substance
- **Transfer a prescription for a narcotic, controlled drug, benzodiazepine or other targeted substance** to another pharmacist. If it has already been transferred, it may be transferred to another pharmacist
- **Refill a prescription for a benzodiazepine or other targeted substance** if more than one year has elapsed since the date it was written
- **Renew** a prescription for controlled substance*
- **Adapt** a prescription for a controlled substance, including part-filling or de-prescribing

*Pharmacists must only prescribe a controlled substance to a patient in accordance with any policies and/or guidelines established by the provincial or territorial government and by any relevant provincial or territorial licensing authorities

EMERGENCY SUPPLY

CONTROLLED SUBSTANCES

The CDSA allows pharmacies/hospitals to purchase an emergency supply of a controlled substances from other pharmacies or hospitals when their supply does not meet the demand

- An emergency supply for a narcotic or controlled drug must be done through a **written order**
- An emergency supply for a benzodiazepine or other targeted substance can be done through **a written or verbal order***
- There must be a record of the purchase from the ordering pharmacy
- There must be a record in the sales report of the sale from the pharmacy selling the substance

The written order of an emergency supply must contain the following information:

- Name and address of the ordering pharmacy
- Name of medication
- Quantity requested (the quantity requested from the ordering pharmacy must only be enough to fill the prescription)
- Signature of the ordering pharmacy

***Note: a written or verbal order for a benzodiazepine or other targeted substance to another pharmacist is allowed if it is due to a delay or mistake in an order placed with a licensed dealer**

PRACTICE QUESTIONS

1. Which medications require either a written or verbal prescription before dispensing (assume no COVID exemptions are in place)?

- a) Nabilone
- b) Amphetamine
- c) Lenoltec #3
- d) Phenobarbital

2. Purchase and Sales records are required for which of the following?

- a) Narcotic drugs
- b) Controlled Drugs Part I
- c) Controlled Drugs Part II
- d) Benzodiazepines and Other Targeted Substances
- e) A and B

3. True or False:

All controlled drugs (Parts I, II and III) require an interval to be specified for refills

PRACTICE QUESTIONS

4. As a federal requirement, which of the following prescriptions would be invalid in two years (assuming no COVID exemptions are in place)?

- a) Morphine
- b) Temazepam
- c) Ramipril
- d) Methylphenidate

5. Which of the following practitioners can prescribe methadone?

- a) Doctor
- b) Midwife
- c) Podiatrist
- d) Chiropodist

METHADONE

Methadone Maintenance Treatment (MMT) is used for the treatment of people who are dependent on opioids

- In the past, physicians required exemptions to prescribe methadone
- As of May 2018, physicians do NOT require exemptions to prescribe methadone (though additional training may still be required as set out by provinces or territories)
- Provinces have independent methadone programs in place regarding methadone prescribing and dispensing
- Urine drug screening (can be random or fixed schedule) is usually done weekly when adjusting the dose during stabilization
- Carry doses: After the patient is stable on methadone, carry privileges differ depending on the physician's discretion
- Daily doses of methadone (except carry doses), should be taken under the direct supervision of a pharmacist or a health care professional

DESTRUCTION OF CONTROLLED SUBSTANCES

WHAT TO RECORD?*

- Names and signatures of 2** witnesses
- Date of destruction
- Method of destruction
- Name of drug
- Strength per unit
- Quantity

HOW LONG DO YOU NEED TO RETAIN YOUR DESTRUCTION RECORDS?

- Minimum of 2 years

WHO DO YOU NEED TO INFORM?

- As of July 2016, you do NOT need to inform or receive approval from Health Canada prior to, or after destruction

HOW TO DESTROY

- Do not pour down drain!
- May choose to dispose via a licensed dealer authorized to destroy controlled substances
- Substance must be rendered so use is deemed impossible or improbable; a change of state is recommended (e.g. solid to a liquid)

* For post-consumer returns, only the date of destruction, method of destruction, unique identifier of the container and number of containers destroyed need to be recorded

** The person carrying out the destruction can be one of the 2 witnesses

DESTRUCTION OF CONTROLLED SUBSTANCES

	Unserviceable Stock	Post-Consumer Returns
Destroyed by:	• Pharmacist • Persons in charge of a hospital (hospitals only) • Licensed health professional (hospitals only; partial or unusable doses and if the drug is already outside the pharmacy, e.g. on a ward)	• Pharmacist
Witnessed by:	• Pharmacist or • Pharmacy intern or • Pharmacy technician or • Practitioner* or • Health Canada inspector (should they be present)	• Pharmacist or • Pharmacy intern or • Pharmacy technician or • Practitioner*

*Practitioner is person who is registered and entitled under the laws of a province to practice in that province the profession of medicine, dentistry or veterinary medicine, and includes any other person or class of persons prescribed as a practitioner

CANNABIS ACT

Designed to better protect the health and safety of Canadians, keep cannabis out of the hands of youth, and to keep profits out of the hands of criminals and organized crime.

- ✓ People 18 or 19 and older (depending on P/T) can **buy, possess or use cannabis** for recreational or medicinal use (those authorized by their HCP)
- ✓ Can **possess or share up to 30 grams of legal dried cannabis, or equivalent** in non-dried form, in public
- ✓ Can **grow** up to 4 cannabis plants per residence for personal use from licensed seeds
- ✓ Legal cannabis products (except products with less than 0.3% THC or no THC) have an **excise stamp** on the package
- ✗ Illegal to take across the Canadian border
- ✗ Serious criminal offence to drive high or work impaired

DRUG SAMPLES

Small quantities provided to pharmacy by pharmaceutical company for free to pharmacists or practitioners

Can pharmacists dispense medication samples to patients?

- Pharmacists may dispense samples if patient is aware
- Samples must be dispensed and labelled in compliance with labelling requirements
- Pharmacists may charge a professional fee (for clinical review and counselling) but NOT the cost of drug
- Schedule I drug samples require a prescription
- Schedule II narcotics **cannot** be provided as samples
- Schedule II, III and unscheduled drug samples do NOT require prescriptions
- If further packaging/labelling is not required, there should be no charge for distributing non-prescription samples

NATIONAL ASSOCIATION OF PHARMACY REGULATORY AUTHORITIES (NAPRA)

- An association of regulatory bodies in Canada
- It's an organization by which provincial and territorial regulators come together to harmonize and discuss provincial/territorial regulations
- NAPRA's recommended drug schedules are published by the National Drug Scheduling Advisory Committee (NDSAC) where NDSAC determines schedules based on risk to public and requirement of pharmacist intervention
- Some provinces/territories do not have their own provincial/territorial drug schedules and instead reference the NAPRA schedules; other provinces review the NAPRA regulations first, and then (almost always) approve them; while others (e.g. Quebec) are more independent and deviate more from NAPRA.
- NDSAC also publishes "Entry to Practice Competencies," the basis of PEBC exams

NAPRA SCHEDULES

Schedule I

- Requires prescription for sale
- Might contain drugs not listed under PDL (F&DA act)
- Must be signed and released by a pharmacist

Schedule II

- Does not require a prescription
- Requires professional intervention by a pharmacist before sale
- Not accessible

Schedule III

- Prescription not required
- Drugs accessible and visible to public but must be within 10 m of dispensary where pharmacist can answer patient questions

Unscheduled (U)

- Sold without professional supervision
- May be sold from any retail outlet
- Labeling is adequate for self selection without supervision and to ensure safety

PROVINCIAL REGULATORY AUTHORITIES

FYI

- Alberta College of Pharmacy (ACP)
- College of Pharmacists of British Columbia (CPBC)
- College of Pharmacists of Manitoba (CPhM)
- New Brunswick College of Pharmacists
- Newfoundland & Labrador Pharmacy Board
- Nova Scotia College of Pharmacists
- Northwest Territories Regulatory Authority
- Ontario College of Pharmacists (OCP)
- Orde Des Pharmaciens du Québec (OPQ)
- Prince Edward Island College of Pharmacy
- Saskatchewan College of Pharmacy Professionals (SCPP)
- Yukon Regulatory Authority (Yukon Consumer Services)

FEDERAL PRIVACY LEGISLATION

PRIVACY ACT

- Only for federal government departments/services
- Protects the privacy of individuals held by a **government institution**

PERSONAL INFORMATION PROTECTION AND ELECTRONIC DOCUMENTS ACT (PIPEDA)

- Encourages personal privacy, confidentiality and access to information and sets out appropriate methods of information collection and retention
- Applies to all **private-sector companies** (including health care corporations such as physicians, pharmacies, etc.)
- Enforced by the Privacy Commissioner of Canada
- If a provincial law is declared “substantially similar” by the Federal Privacy Commissioner, then the provincial law takes effect

Since PEBC exams are federal exams, provincial privacy laws won't be tested

PIPEDA

10 PRINCIPLES TO SAFEGUARD CONFIDENTIALITY

1. **Accountability:** Responsible party within the organization
2. **Identifying purpose:** i.e. Why is the information collected
3. **Consent:** Obtained when appropriate
4. **Limiting collection:** Only vital data collection
5. **Limiting use, disclosure, and retention:** Only used when and where necessary
6. **Accuracy:** Updated and complete information
7. **Safeguards:** Appropriate control of information
8. **Openness:** Privacy policy statements accessible
9. **Individual access:** You have a right to access your own information
10. **Challenging compliance:** You have the right to submit corrections

CONFIDENTIALITY

DISCLOSURE OF CONFIDENTIAL INFORMATION

Disclosure required

- Child or elderly abuse
- Reportable communicable diseases (e.g. HIV)
- Gunshot wounds

Disclosure permitted

- Patient consent provided
- Substitute decision maker (SDM) or guardian consents if patient is incapacitated
- Medical professionals within patient's circle of care
- Patients at risk of self-harm or of harming others
- Court orders
- Gunshot wounds

Disclosure will not likely be tested on PEBC exam due to variations between provinces. However, you should understand the overall concept of disclosure.

TAKEAWAY

FEDERAL ACT	FOCUSES ON
F&DA	• Food, drugs, medical devices, and cosmetics • Relevant regulations – part C (PDL) and part G (controlled drugs)
CDSA	• Narcotics and benzodiazepines and targeted substances • Specifies who can prescribe controlled substances (doctors of medicine, dentists, doctors of veterinary medicine, midwives, podiatrists and nurse practitioners)
Cannabis act	Recreational cannabis use
Privacy Act	Privacy of individuals held by government institutions
PIPEDA	Privacy of information collected by private sector companies

- Methadone is regulated provincially
- Narcotics and controlled substances are usually destroyed by a pharmacist and witnessed by either a pharmacist, pharmacy intern, pharmacy technician or a practitioner
- NAPRA drug schedules (I,II,III and U) help with placement and sale of drugs based on risk to public and requirement of pharmacist intervention

PRACTICE CASE

1. Which of the following examples shows a legally correct refill designation on a written prescription for phenobarbital 60 mg PO BID x 3/52?
 - a) Repeat twice for 6 weeks
 - b) Repeat monthly
 - c) Repeat every 21 days for 9 weeks
 - d) Repeat one time

2. According to federal legislation, which of the following requirements is CORRECT for a verbal prescription of sertraline?
 - a) Verbal prescriptions are NOT allowed
 - b) An interval is required for refills
 - c) Only one transfer is permitted
 - d) Sales report is NOT required

PRACTICE CASE

3. A patient comes into the pharmacy complaining of pain and informs you that her doctor has faxed in her prescription for Lenoltec #1 (acetaminophen 300 mg, caffeine 15 mg, codeine 8 mg). The prescription is for 90 tablets, but you only have 30 in stock. The patient requests that you transfer her prescription to another pharmacy. What is the MOST appropriate response?
- a) Transfer the prescription as requested
 - b) Suggest the patient purchase some over-the-counter acetaminophen 500 mg with caffeine 65 mg tablets until you receive additional stock
 - c) Call the patient's doctor to have her prescription sent to another pharmacy
 - d) Dispense the 30 tablets available and give the patient a refill prescription to fill another pharmacy

PRACTICE CASE

4. You report for your shift and your pharmacy assistant informs you that there is some expired stock of narcotics that needs to be destroyed. Included are 20 hydrocodone 5 mg tablets. Which of the following information is NOT required to proceed with the destruction?
- a) Date
 - b) 3 Witnesses
 - c) Quantity
 - d) Drug name

USEFUL RESOURCES



Health Canada website



NAPRA



Prescription Regulation Summary Chart



Ontario College of Pharmacists



Narcotic Control Regulations (Refer to schedule at the end for list of narcotics)



F&DA, list of controlled drugs



Benzodiazepines and Other Targeted Substances, list

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CHANGE LOG

March 2025

- Slide 3: Updated legend
- Slide 32: Corrected some college of pharmacy names
- Slide 40: Updated last link for benzodiazepines and controlled substances
- Slide 41: Added CPhA reference used for slide 32

August 2025

- Slides 15 and 20: Changed 'straight narcotics' to narcotic drugs
- Slides 17 & 18: Minor edits to definitions
- Slide 26: Updated instructions under 'How to Destroy'
- Slide 35: Removed 'Reportable communicable diseases (e.g. HIV)' from disclosure permitted

November 2025

- Formatting changes made throughout
- Slide 8: added Part G examples
- Slide 26: Added note on witnesses

February 2026

- Slide 2: Updated NAPRA competencies
- Slide 10: Changed cocoa leaves to "coca"
- Slide 15: removed mention of dextropropoxyphene (withdrawn)
- Slide 19: Added zolpidem as an example
- Slide 24: Updated question 4